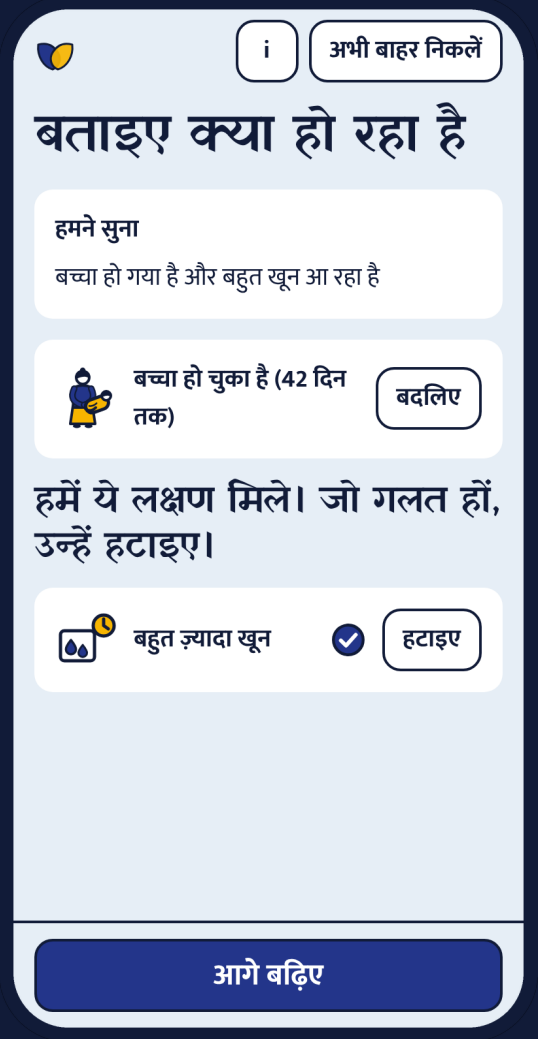
Maitri

The friend who answers when it matters most.

Voice-first maternal emergency guidance for every family

Speak it. See why. Call at once.





Team Decyphers

Aafreen Khan (team leader) | Aastha Pandya | Prisha Sharma

recHERsion 2026 | Problem Statement 1: Women's Healthcare

11:40 pm

Six hours after the birth, the bleeding has not stopped.

Her husband holds the phone. Her mother-in-law says wait for morning. Nobody in the room knows if this is normal.

Illustrative scene, not a real person.

25.2M

births a year in India

Secondary source

48.6%

of maternal deaths in one hospital study involved a delay in deciding or seeking care

Verified, single study

1 in 3

women has fewer than four antenatal visits

Secondary, derived

THE PROBLEM

A warning sign appears and the family cannot tell if it is routine or an emergency. They wait, argue and call the wrong person. Those lost minutes, before anyone has asked for help, are what we solve.

Why today's options fail the family

	Paper card	Helpline	Symptom chatbot	Maitri
Works with no internet	✓	✗	✗	✓
Hindi: listens and answers aloud	✗	✓	–	✓
Tells the family the next step	–	✓	–	✓
Shows why, with its sources	✓	✗	✗	✓
Hands a summary to the ASHA or doctor	✗	✗	✗	✓

Typical examples and our own assessment from limited searching, not every product. A helpline can give advice in Hindi; it needs a phone, signal and a clear head.

Meet Maitri

Say it. See why. Know what to do.

SAY IT

Speak or tap, in Hindi or English. Hear every question and result read aloud.

SEE WHY

Every result names the rule and the source. No AI decides.

KNOW WHAT TO DO

One RED sign ends the check. One call, one plan, one slip.

Five things only Maitri does, together

1. Voice in, voice out
2. The call comes first
3. It shows its work
4. From panic to a plan
5. Private and offline

USP 1 + 2

Voice in, voice out. The call comes first.

Built for a family that cannot type in a crisis and should never have to look for the ambulance number



Honest note: speech recognition needs internet on most phones. Spoken output uses the phone's own voice.

1 Speak it

One tap, then say what is happening in Hindi or English. Speech-to-text turns it into the signs.

2 Check and edit

She sees what was heard and can remove a wrong sign. A spoken "No" is confirmed first.

3 One RED sign, one call

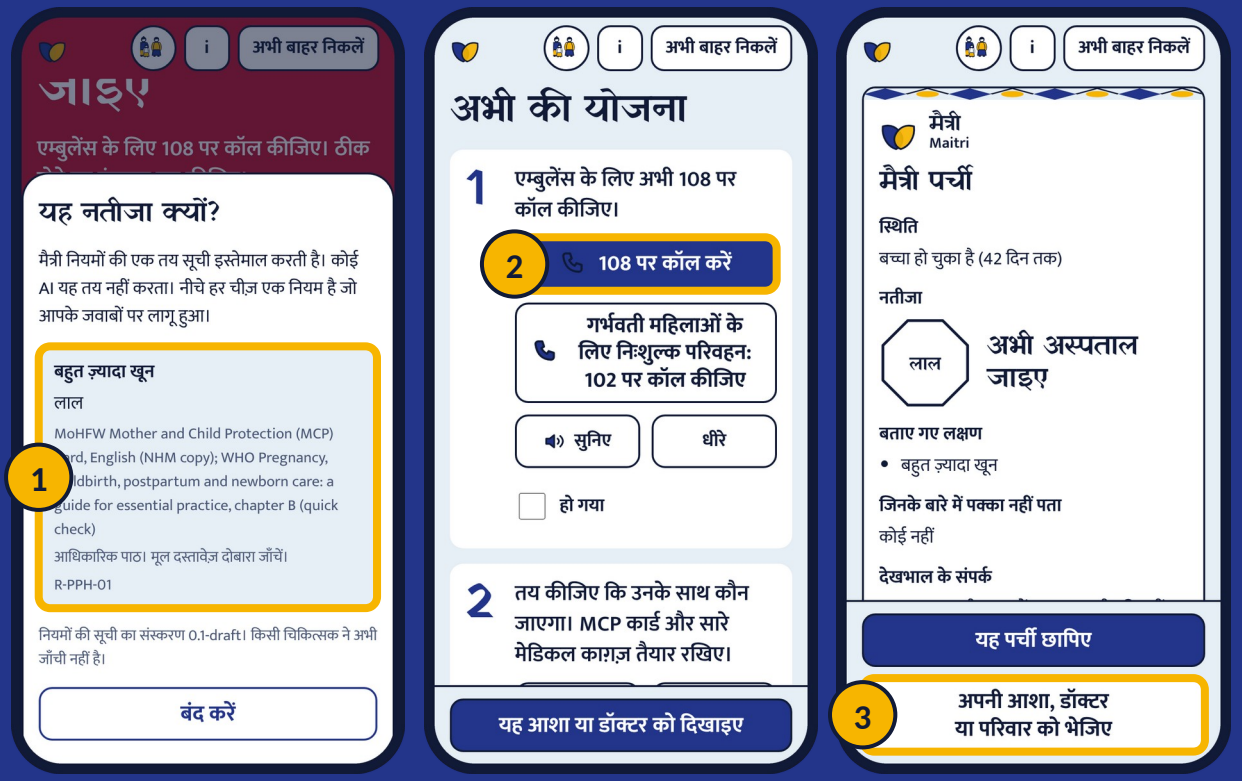
The check stops at once. The call button is the largest control. Never "fine", and "not sure" goes up.

Listen and Slow read every question and result aloud (text-to-speech). The emergency call is the first control on the first screen.

USP 3 + 4

It shows its work. It gives a plan.

The whole reasoning on screen in one tap, and the family leaves knowing who to call and what to hand over



Rules are a draft and not yet clinician reviewed. The tests check the code matches the rules; they are not clinical validation.

1 "Why this result?"

The rule, the sign, the source and the rule version. A fixed list a clinician can review. No AI decides.

2 The go-now plan

Call, decide who goes, show the slip, ask for free transport and care (JSSK). Steps to tick off.

3 The Maitri slip

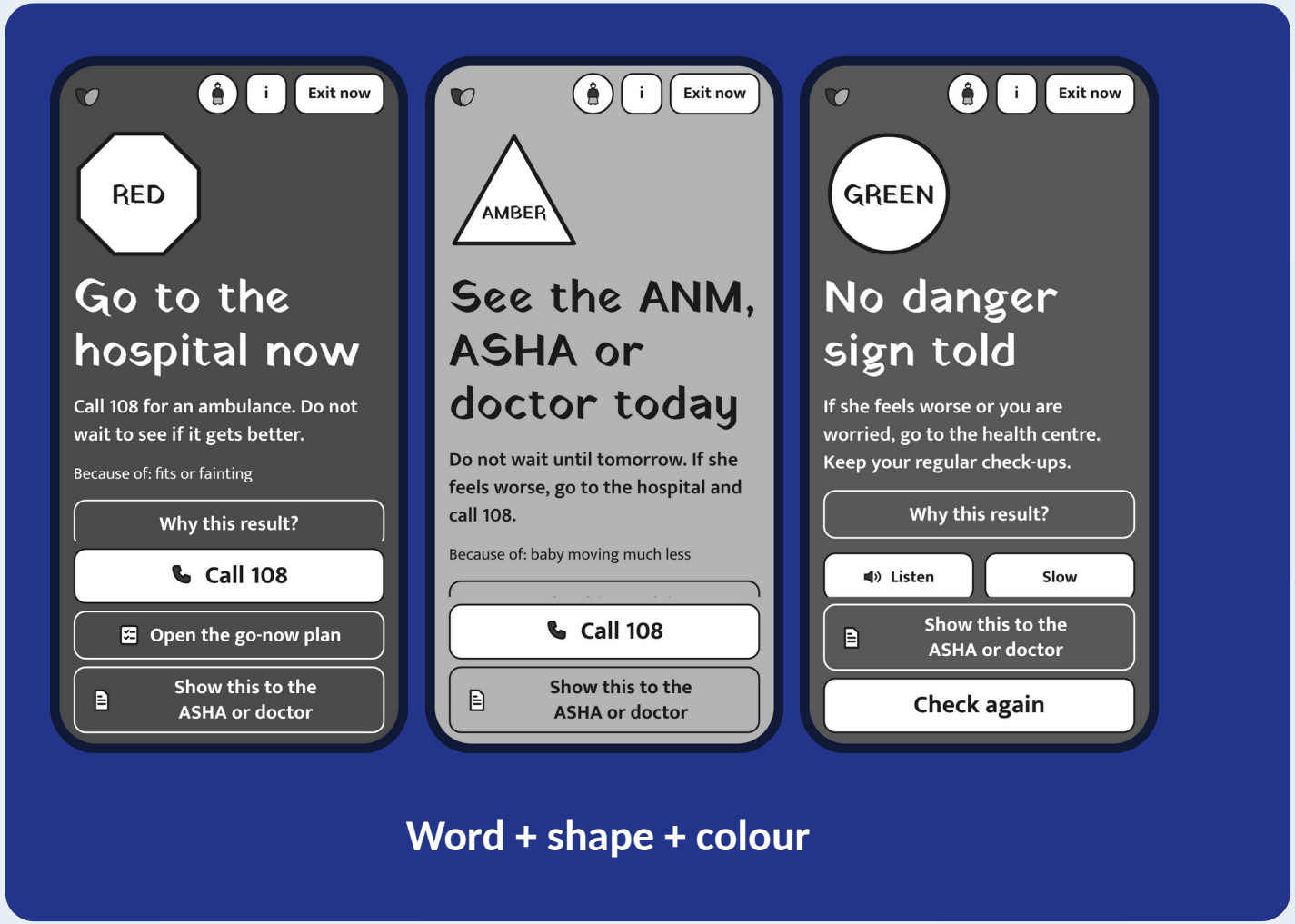
Stage, result and signs told. No name, no phone number. Shared only after she agrees.

205	46	12,420
automated tests	rule cases	cross-checked

USP 5

Private, offline, for everyone.

It still reads in greyscale, in bright sun, on a cheap phone, with no signal



Works offline

Verified in a browser with the network cut. Installs like an app.

Private by default

No account needed. "Exit now" clears everything and leaves.

Hindi and English, audited

A test fails if any screen shows the wrong language.

Pictures and big buttons

One question per screen, tick and cross, large targets, axe audit.

Built to be trusted. Honest about the gaps.

205	46	12,420	15	0
tests pass	rule cases checked	cases re-checked in Python	screens audited at 4 sizes	clinician reviews yet

Works today

- Tap and voice check, Hindi and English
- RED, go-now plan, slip, Why
- Offline, private, Trust page

Needs a real test

- Voice on real phones
- Sign-in and daily log (Supabase)
- Sharing on real phones

Not done yet

- Clinician review
- Recorded voice clips
- Field test with ASHAs

How it is built: React, TypeScript and Vite. Voice is the browser's speech-to-text, then word lists, then the same Yes / No / Not sure, then a fixed rule engine. No model decides. Optional Supabase cloud with row level security: built and tested locally, not connected yet.

Feasible now. A clear path. Sustainable.

Static site
No server for the check

Offline
Verified, network cut

Low cost
About INR 6 per language for clips

Privacy first
No account needed

Now
Clinician review. ASHA field test. Real-phone voice test. Recorded voice clips.

Next
Odia, Assamese, Bengali with native review. Connect sign-in and daily log.

Then
Visit and vaccine schedules and scheme help, each with an official source.

Later
Facility finder from a verified dataset.

State missions and NGOs
Licence, localisation, ASHA training

CSR and grants
Fund languages, clips, clinician review

Open core
Rules and app stay open. Services are paid.

Free for women and families. We never sell data. No ads. A proposed funding model, not yet tested with a funder.

Every minute counts.

Maitri makes the next one clear.

Speak it. See why. Call at once.

To take it from prototype to pilot, we need three things

A clinician reviewer

To check and sign off the rules

An ASHA district

To pilot with real families

Language partners

Odia, Assamese, Bengali

Scan for code and video

Team Decyphers | Aafreen Khan (team leader) | Aastha Pandya | Prisha Sharma

Education tool. Does not diagnose. Rules are a draft, not yet reviewed by a doctor. Sources: docs/PITCH_FACTS.md.